

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<think> The symptoms described include hyperactivity, short attention span, disruptive behavior, and forgetting tasks, which suggest ADHD (Attention-deficit/hyperactivity disorder). The mother has a mental health issue (major depressive disorder), and the father has a thyroid condition (Graves' disease), which could contribute to these symptoms. Behavior therapy is a common first-line treatment for ADHD. Methimazole is used for thyroid conditions, and fluoxetine is used for depression, not ADHD. Hearing aids are not typically used for ADHD.

</think>
<answer>A. Behavior therapy</answer>

